Emergency Department Note

SYNTHETIC DOCUMENT - generated for software testing, not a real patient record

Case ID	SYN-0012	**Age / Sex**	58 / M
Setting	Emergency Dept	**Primary dx**	Hepatic encephalopathy grade III

Chief complaint

Progressive confusion and sleepiness over 3 days.

History of present illness

Wife reports day-night reversal, then slurred and nonsensical speech, now difficult to wake. Constipated for 5 days, recent melena. Continued drinking until a week ago. Not taking lactulose for the last 2 weeks.

Past history / medications

Alcohol-related cirrhosis (Child-Pugh C), previous variceal bleed, ascites. Propranolol, spironolactone, lactulose (non-adherent).

Examination

HR 98, BP 102/60, afebrile. Jaundice, spider naevi, tense ascites, asterixis. Pupils equal and reactive. Hyperreflexia. No focal deficit or meningism.

Glasgow Coma Scale

Eyes open only to voice. Speech limited to inappropriate, unrelated words. Motor: localizes painful stimuli but does not follow commands.

GCS on arrival	Score	Response
Eye opening (E)	3	To voice
Verbal (V)	3	3 - Inappropriate words
Motor (M)	5	Localizes pain
TOTAL GCS		**11**

GCS at 36 h (after lactulose)	Score	Response
Eye opening (E)	4	Spontaneous
Verbal (V)	5	5 - Oriented
Motor (M)	6	Obeys commands
TOTAL GCS		**15**

Investigations

Ammonia 148 umol/L, bilirubin 112 umol/L, INR 1.9, albumin 24 g/L, sodium 129. Hb 74 g/L with positive fecal occult blood. Diagnostic paracentesis: PMN 120 per mm3, no spontaneous bacterial peritonitis. CT head normal.

Assessment

Hepatic encephalopathy grade III (West Haven) triggered by upper GI bleeding, constipation, and lactulose non-adherence. GCS 11.

Plan / disposition

Lactulose enemas then oral, rifaximin, transfusion to Hb above 70 g/L, IV terlipressin and urgent endoscopy for variceal bleeding, antibiotic prophylaxis. GCS 15 at 36 h. Admitted to hepatology.